

HealthRAG AI - Health Reference Guide

This reference summarises common health conditions and first-aid guidance for educational use within the HealthRAG knowledge base. It is not a substitute for professional medical advice. Always consult a qualified clinician for diagnosis and treatment.

Migraine

Migraine is a common neurological condition causing recurrent moderate to severe headaches, typically one-sided and throbbing, lasting 4 to 72 hours. Attacks are often accompanied by nausea, vomiting, and sensitivity to light and sound. Some people experience an aura before the headache, such as flashing lights or zig-zag lines, tingling, or speech disturbance. Common triggers include stress, missed meals, dehydration, poor sleep, hormonal changes, alcohol, and certain foods. Acute treatment includes rest in a dark quiet room, simple analgesics such as ibuprofen or aspirin, and triptans such as sumatriptan for moderate to severe attacks. Anti-sickness medication can help. If attacks are frequent, preventive medication such as propranolol, amitriptyline, or topiramate may be offered, along with a headache diary to identify triggers. Seek urgent care for a sudden severe thunderclap headache, headache with fever and neck stiffness, new headache after age 50, or headache with weakness, confusion, or visual loss.

Iron-deficiency anaemia

Iron-deficiency anaemia occurs when the body lacks enough iron to make adequate haemoglobin, reducing the oxygen-carrying capacity of the blood. Symptoms include tiredness, weakness, shortness of breath on exertion, pale skin, headaches, brittle nails, and sometimes unusual cravings for non-food items such as ice. Common causes are blood loss from heavy periods or the gastrointestinal tract, poor dietary intake, pregnancy, and malabsorption conditions such as coeliac disease. Diagnosis uses a full blood count showing low haemoglobin and low mean cell volume, with a low ferritin confirming low iron stores. Treatment is oral iron supplements, usually ferrous sulfate or ferrous fumarate, taken with vitamin C to aid absorption and away from tea, coffee, and calcium. Haemoglobin should rise within a few weeks and treatment continues for about three months after levels normalise to rebuild stores. The underlying cause must be investigated, especially in men and postmenopausal women, where gastrointestinal bleeding must be excluded.

Urinary tract infection

A urinary tract infection (UTI) is a bacterial infection of the bladder (cystitis) or, less commonly, the kidneys (pyelonephritis). Bladder infection causes burning or stinging when passing urine, needing to urinate more often and urgently, lower abdominal discomfort, and cloudy or strong-smelling urine. Kidney infection adds fever, chills, back or flank pain, nausea, and feeling generally unwell, and needs prompt medical assessment. Most uncomplicated bladder infections in women are treated with a short course of antibiotics such as nitrofurantoin or trimethoprim, guided by local resistance patterns, along with drinking plenty of fluids and paracetamol for pain. Seek medical advice if symptoms are severe or do not improve within 48 hours, if there is blood in the urine, fever, or back pain, if the person is pregnant, male, a child, or has a catheter or recurrent infections.

Gastro-oesophageal reflux disease

Gastro-oesophageal reflux disease (GORD) is caused by stomach acid flowing back into the oesophagus. Typical symptoms are heartburn, a burning feeling rising from the stomach to the chest, an acid or bitter taste in the mouth, and regurgitation, often worse after meals, when lying down, or bending over. It can also cause a chronic cough, hoarseness, or a sensation of a lump in the throat. Self-care measures include eating smaller meals, avoiding known triggers such as fatty or spicy food, coffee, alcohol, and chocolate, not eating within three hours of bed, raising the head of the bed, losing excess weight, and stopping smoking. Antacids and alginates relieve occasional symptoms; a proton pump inhibitor such as omeprazole is used for frequent or persistent symptoms. Refer for endoscopy if there is difficulty swallowing, unintentional weight loss, persistent vomiting, gastrointestinal bleeding, or symptoms that do not respond to treatment.

Hypothyroidism

Hypothyroidism is an underactive thyroid gland that does not produce enough thyroid hormone, slowing the body's metabolism. Symptoms develop gradually and include tiredness, weight gain, feeling cold, constipation, dry skin and hair, low mood, poor concentration, muscle aches, and heavy or irregular periods. The most common cause is autoimmune thyroiditis. Diagnosis is by blood tests showing a raised thyroid-stimulating hormone (TSH) with a low free thyroxine (T4). Treatment is once-daily levothyroxine, taken on an empty stomach, with the dose adjusted according to repeat TSH testing every 6 to 8 weeks until stable, then annually. Symptoms improve over weeks to months. Pregnancy increases thyroxine requirements and needs closer monitoring.

Allergic rhinitis

Allergic rhinitis is inflammation of the lining of the nose triggered by allergens such as pollen (hay fever), house dust mites, or animal dander. Symptoms are sneezing, an itchy or runny nose, nasal blockage, and itchy watery eyes, and it can worsen asthma. Management starts with allergen avoidance where possible. Non-sedating antihistamines such as cetirizine or loratadine relieve sneezing, itch, and runny nose. A regular intranasal corticosteroid spray such as fluticasone is the most effective treatment for nasal blockage and moderate to severe symptoms and works best when started before the pollen season and used daily. Saline nasal rinses help. Persistent symptoms despite treatment may be referred for specialist assessment and immunotherapy.

Low back pain

Non-specific low back pain is very common and usually improves within a few weeks. It is felt in the lower back and can spread to the buttocks or thighs, often triggered by lifting, awkward movements, or prolonged poor posture. Most cases do not need imaging. Advice is to stay active and continue normal activities as much as pain allows, avoid bed rest, use heat, and take a short course of an anti-inflammatory such as ibuprofen if suitable. Gentle exercise, stretching, and physiotherapy help recovery and prevent recurrence. Seek urgent assessment for red-flag features: numbness around the buttocks or genitals, loss of bladder or bowel control, progressive leg weakness, unexplained weight loss, fever, a history of cancer, or pain following significant trauma.

General first aid: choking in an adult

If an adult is choking and cannot cough, speak, or breathe, stand behind them and give up to five sharp back blows between the shoulder blades with the heel of your hand. If that fails, give up to five abdominal thrusts by placing a fist just above the navel, grasping it with the other hand, and pulling sharply inward and upward. Alternate five back blows and five abdominal thrusts. Call emergency services if the obstruction does not clear. If the person becomes unresponsive, lower them to the ground and start CPR.

General first aid: recovery position

Place an unconscious person who is breathing normally in the recovery position to keep the airway open and allow fluids to drain. Kneel beside them, place the near arm at a right angle, bring the far arm across the chest with the back of the hand against the near cheek, bend the far knee, and roll the person toward you onto their side. Tilt the head back slightly, adjust the upper leg so hip and knee are bent at right angles, and monitor breathing until help arrives. Do not use the recovery position if a spinal injury is suspected unless the airway cannot otherwise be maintained.